



MEDICATION ADMINISTRATION MANUAL IMPLEMENTATION CHECKLIST BINDER

Companion to ECC-MED-2026-001, Version 1.0 — Effective July 1, 2026

Document Reference	ECC-MED-2026-001-IMP (companion to ECC-MED-2026-001, Version 1.0)
Program Name	
Manual Effective Date	July 1, 2026
Program Manager (PM)	
Program Coordinator (PC)	
Binder Prepared By / Date	
Target Full Implementation	Jul 31, 2026 (30 days from effective date)
Related Documents	RC Program Manual v4 (ECC-RC-POL-2026-003); Behaviour Support Manual (ECC-BSP-2026-001); OHS Program Binder (ECC-OHS-POL-2026-002); HR Policy Binder v3 (ECC-HR-POL-2026-001); PC Manual (ECC-PC-MAN-2026-001); PM Manual (ECC-PM-MAN-2026-001)

How to Use This Binder

This binder converts the Medication Administration Manual (ECC-MED-2026-001) into role-based implementation checklists with due dates. Due dates are calculated from the manual effective date of July 1, 2026 — adjust in ink if your program's timeline differs.

- Part 1 — Program Manager (PM): authorization, oversight, and governance tasks.
- Part 2 — Program Coordinator (PC): set-up, documentation, storage, training delivery, and audits.
- Part 3 — Youth Care Workers & Shift Leads: per-staff readiness. Photocopy Part 3 for each staff member — Section A must be 100% complete before that person's first medication duty.
- Part 4 — Recurring Compliance Schedule: the ongoing cycles that continue after initial implementation.
- Check the box, then enter the date and your initials in the Completed column as each item is finished. Retain the completed binder as evidence of implementation for licensing and MCFD review.

Contents

Part 1 — Program Manager (PM) Implementation Checklist

Owner: Program Manager. 23 tasks. **No staff member administers any medication until Section B is complete for that person.**

✓	#	Task	Manual Ref	Due / Frequency	Completed (date / initials)
A. Governance & Approval (Week 1)					
☐	1	Confirm ED signature on the manual and file the signed copy; distribute manual to program	Approval p.; §1	Jul 3, 2026	
☐	2	Read the full manual; brief Shift Leads and PC on key changes at a staff meeting <i>Document the meeting and attendance</i>	All Parts	Jul 8, 2026	
☐	3	Update cross-references to ECC-MED-2026-001 in: RC Program Manual v4 (ECC-RC-POL-2026-003); PC Manual (ECC-PC-MAN-2026-001); PM Manual (ECC-PM-MAN-2026-001) <i>Also review Behaviour Support Manual (ECC-BSP-2026-001), OHS Binder (ECC-OHS-POL-2026-002), HR Policy Binder v3 (ECC-HR-POL-2026-001) for medication references</i>	Related Documents	Jul 15, 2026	
☐	4	Resolve version discrepancy in the manual: header/version history state Version 1.0 but the page footer reads 'Version 2.0 July 2026' — correct the footer and re-issue <i>Footer and version history must match before distribution</i>	Doc control	Jul 8, 2026	
B. Staff Authorization (Weeks 1–3) — no medication duty until complete					
☐	5	Verify current MALM (or equivalent) certificate for every staff member expected to administer medications <i>SHSS Standard G.3 — training before any medication duty</i>	§4.1	Jul 8, 2026; Then annual	
☐	6	Remove any staff without current training/authorization from all medication duties immediately	§4.1	Jul 3, 2026; Immediate	
☐	7	Observe or delegate competency demonstrations (Seven Rights, MAR, CDS count, error reporting) <i>PC may assess; PM issues authorization</i>	§4.1–4.2	Jul 15, 2026; Then annual	
☐	8	Issue written program-specific authorization for each eligible staff member (Appendix A) <i>Copy to personnel file + program authorization binder</i>	§4.1; App A	Jul 15, 2026; Then annual	
☐	9	Record CDS, EpiPen, and Naloxone authorization status on each Appendix A form	App A	Jul 15, 2026	
☐	10	Set authorization expiry tracking (12 months from issue) with 60-day renewal reminders	§4.1	Jul 22, 2026; Ongoing	
C. Clinical & External Relationships (Weeks 2–4)					
☐	11	Establish/confirm the primary pharmacy relationship; post pharmacy contacts in the medication area <i>Request blister pack dispensing where appropriate</i>	§40; App E	Jul 15, 2026	
☐	12	Compile prescriber contact list for all youth, including after-hours contacts	§40; App E	Jul 15, 2026; Review monthly	
☐	13	Approve a written PRN protocol for every PRN medication (drafted by PC) <i>Target symptom, dose, max frequency, monitoring, escalation</i>	§18	Jul 22, 2026; Review annually	
☐	14	Confirm MCFD Category 1/2 CI notification pathway for medication errors	§29; §3	Jul 15, 2026	
D. CDS & Error System Oversight (Weeks 2–4)					
☐	15	Verify double-locked CDS storage and separate key-holding arrangements at the program	§10	Jul 8, 2026; Then	

✓	#	Task	Manual Ref	Due / Frequency	Completed (date / initials)
				monthly	
☐	16	Review the CDS register / count sheets; sign the first monthly reconciliation	§10–11; App C	Jul 31, 2026; Monthly	
☐	17	Confirm error reporting workflow: PC reports within 2 hrs; PM reviews and co-signs within 24 hrs	§29–30	Jul 15, 2026; Ongoing	
☐	18	Confirm Naloxone kit on site and staff trained wherever any youth is prescribed opioids	§23.3	Jul 15, 2026; Renew per expiry	
E. Ongoing Oversight (first cycle dates)					
☐	19	Review PC monthly MAR audit summary (by the 15th of each month) <i>First cycle: Aug 15, 2026 for July MARs</i>	§38.1	Aug 15, 2026; Monthly	
☐	20	Review controlled substance register	§3; §10	Aug 1, 2026; Monthly	
☐	21	Complete root cause analysis within 5 business days of any error; log corrective actions	§31	Per event	
☐	22	Submit quarterly medication incident trend report to ED <i>First report: end of Q3 2026</i>	§3; §31	Oct 1, 2026; Quarterly	
☐	23	Annual manual review and re-approval; re-issue all staff authorizations <i>July 1, 2027</i>	§4.1	Jul 1, 2027; Annual	

Part 2 — Program Coordinator (PC) Implementation Checklist

Owner: Program Coordinator. 29 tasks covering documentation set-up, storage and security, clinical protocols, training delivery, and ongoing duties.

✓	#	Task	Manual Ref	Due / Frequency	Completed (date / initials)
A. Documentation Set-Up (Week 1)					
☐	1	Initiate a current-month MAR for every youth with active medications (Appendix B or pharmacy MAR) <i>One MAR per youth per calendar month</i>	§6.1; App B	Jul 4, 2026; Then monthly	
☐	2	Create/update the Individual Medication Profile for every youth — allergies prominent on every page <i>Include PRN, OTC, discontinued (12 mo), interactions</i>	§7	Jul 8, 2026; Review monthly	
☐	3	Set up CDS Count Sheets for every controlled substance in the cabinet (Appendix C)	§10–11; App C	Jul 4, 2026; Then per shift	
☐	4	Print and post the Poison Control / Emergency Reference card in medication and staff areas (Appendix E) <i>1-800-567-8911 visible</i>	App E	Jul 4, 2026	
☐	5	Stock blank forms: Error Report (D), PRN Record (F), Disposal (G), Refusal (H), Acknowledgement (I)	App D–I	Jul 8, 2026; Restock as needed	
☐	6	Verify consent documentation on file for every medication (MCFD, prescriber, OTC authorizations)	§5	Jul 11, 2026; Per change	
B. Storage & Security (Weeks 1–2)					
☐	7	Verify medication cabinet: locked, solid, secured, inaccessible to youth; one key per shift	§9	Jul 4, 2026	
☐	8	Verify CDS double-lock: locked box inside locked cabinet; inner key held separately by senior staff	§10	Jul 4, 2026	
☐	9	Place thermometers: cabinet (15–25°C) and medication fridge with min/max (2–8°C); start temperature logs <i>Fridge logged daily when refrigerated meds in use</i>	§9	Jul 8, 2026; Monthly/daily	
☐	10	Set up key log; confirm all cabinet and CDS keys accounted for; establish shift-change key transfer	§9–10	Jul 4, 2026; Then per shift	
☐	11	Confirm each youth's medications stored separately, in original containers with original labels	§9	Jul 8, 2026; Then monthly	
☐	12	Verify EpiPen (if applicable): in date, room temperature, NOT locked away, location known to all staff	§23.2	Jul 4, 2026; Then monthly	
☐	13	Conduct baseline full inventory: count all medications, check expiry, reconcile to MAR and profiles <i>Remove and dispose of expired stock (§33)</i>	§9; §32.1	Jul 8, 2026; Then monthly	
C. Clinical Protocols (Weeks 2–3)					
☐	14	Draft written PRN protocols for every PRN medication and submit to PM for approval	§18	Jul 15, 2026; Once per PRN	
☐	15	Obtain standing OTC authorizations from prescriber/MCFD (acetaminophen, ibuprofen, etc.) <i>Document source and date on the profile</i>	§19	Jul 15, 2026; Renew annually	
☐	16	Document self-administration authorizations (incl. rescue inhaler exception) where applicable <i>Prescriber + PM approval; MCFD awareness</i>	§20	Jul 15, 2026; Review per youth	

✓	#	Task	Manual Ref	Due / Frequency	Completed (date / initials)
☐	17	Build the medication renewal calendar; renewals initiated 2+ weeks before expiry	§32.2	Jul 15, 2026; Ongoing	
☐	18	Record special monitoring requirements (lithium, lamotrigine, antipsychotics, insulin, etc.) in each profile	§24	Jul 15, 2026; Per change	
D. Staff Training Delivery (Weeks 2–4)					
☐	19	Deliver in-person modules: MAR (3), CDS (4), storage walkthrough (5), errors (6), special meds (7), youth rights (8) <i>MALM online completed by staff first</i>	§4.2	Jul 22, 2026; Then annual	
☐	20	Assess and document competency demonstrations; forward results to PM for authorization	§4.1	Jul 22, 2026; Then annual	
☐	21	Collect signed Appendix I acknowledgement from every authorized staff member <i>Before first medication duty</i>	App I	Jul 22, 2026; Then annual	
☐	22	Build the medication authorization binder (Appendix A copies, MALM certificates, acknowledgements)	§4.1	Jul 22, 2026; Maintain ongoing	
E. Ongoing PC Duties (first cycle dates)					
☐	23	Weekly MAR review for completeness, accuracy, anomalies	§3	Jul 8, 2026; Weekly	
☐	24	Weekly PRN usage review; flag any PRN used 3+ times in 7 days to PM and prescriber	§18; App F	Jul 8, 2026; Weekly	
☐	25	Monthly medication review checklist (inventory, expiry, labels, storage, PRN records) <i>By last business day; submit to PM within 5 business days</i>	§32.1	Jul 31, 2026; Monthly	
☐	26	Monthly MAR audit for every youth — by the 10th of the following month <i>First audit: July MARs by Aug 10, 2026</i>	§38.1	Aug 10, 2026; Monthly	
☐	27	Action new medication orders within 24 hours of receipt	§3	Per event	
☐	28	Report all medication errors to PM within 2 hours; ensure Appendix D completed before end of shift	§3; §29–30	Per event	
☐	29	Prepare medication summary before every prescriber appointment; action changes same day	§32.2	Per appointment	

Part 3 — Youth Care Worker & Shift Lead Readiness Checklist

Photocopy this Part for each staff member. Section A is the readiness gate — every item must be complete, verified by the PC, before this staff member's first medication duty. Sections B and C describe ongoing per-shift obligations acknowledged at sign-off.

Staff Member Name	
Position	
Program	
MALM Certificate Expiry	
Authorization Expiry (12 months from issue)	

✓	#	Task	Manual Ref	Due / Frequency	Completed (date / initials)
A. Before First Medication Duty — required for EVERY staff member (no exceptions)					
☐	1	Complete BC MALM (or approved equivalent) online modules; provide certificate to PC	§4.1–4.2	Jul 15, 2026; Renew annually	
☐	2	Complete in-person modules with PC: MAR, CDS count, storage walkthrough, errors, special meds, youth rights	§4.2	Jul 22, 2026; Then annual	
☐	3	Pass competency demonstration: Seven Rights, MAR documentation, CDS count, error reporting	§4.1	Jul 22, 2026; Then annual	
☐	4	Demonstrate each administration route: oral, topical, inhaler/nebulizer, eye/ear drops <i>Return demonstration for each route</i>	§14–17	Jul 22, 2026	
☐	5	Complete EpiPen training on trainer device; Naloxone training if opioids on site	§23	Jul 22, 2026; Refresh annually	
☐	6	Read the full manual and sign the Staff Acknowledgement (Appendix I)	App I	Jul 22, 2026; Re-sign annually	
☐	7	Receive written program-specific authorization from PM (Appendix A) — DO NOT administer before this <i>Authorization is program-specific and non-transferable</i>	§4.1; App A	Jul 22, 2026; Renew at 12 months	
☐	8	Know the location of: Poison Control card, EpiPen, Naloxone kit, on-call schedule, pharmacy number	App E; §23	Jul 22, 2026	
B. Every Shift — all authorized YCWs					
☐	9	Verify the Seven Rights before EVERY dose — never pre-sign the MAR <i>Pre-signing is a serious medication error</i>	§12	Every dose	
☐	10	Document on the MAR immediately at administration (initials + actual time), or code R / H / NA / S	§6.2; §13	Every dose	
☐	11	Participate in the CDS two-person count at shift change; two staff at every CDS administration (one administers, one witnesses); sign the count sheet <i>Discrepancies investigated before outgoing shift departs</i>	§10–11; §21.1; App C	Every shift change	
☐	12	Keep the medication cabinet locked at all times except during authorized administration	§9	Continuous	
☐	13	Report any error or near-miss immediately to PC/on-call; complete Appendix D before end of shift <i>Poison Control 1-800-567-8911 if any uncertainty</i>	§28–30; App D	Per event	
☐	14	Handle refusals therapeutically — never force,	§25; App H	Per event	

✓	#	Task	Manual Ref	Due / Frequency	Completed (date / initials)
		coerce, or give covertly; complete Appendix H			
☐	15	Observe and document medication effects and side effects in the shift log throughout the shift	§35	Every shift	
C. Shift Lead — additional duties					
☐	16	Verify medication cabinet locked at every shift; confirm key transfer and key log entry	§3; §9	Every shift	
☐	17	Supervise the CDS count at every shift change; escalate any discrepancy immediately	§3; §11	Every shift change	
☐	18	Confirm all MAR entries for the shift are complete before the outgoing shift departs	§3	Every shift change	
D. Annual (per staff member)					
☐	19	Annual competency review with PC/PM (Seven Rights, MAR, CDS count, error scenario) <i>Before authorization expiry</i>	§4.1–4.2	Jun 16, 2027; Annual	
☐	20	Renew MALM certificate and PM re-authorization before the 12-month expiry	§4.1	Jun 16, 2027; Annual	
☐	21	Re-sign Appendix I on each annual manual update	App I	Jul 1, 2027; Annual	

Sign-Off

Staff Member Signature / Date	
PC Signature (Section A verified complete) / Date	
PM Signature (Authorization issued — Appendix A) / Date	

Part 4 — Recurring Compliance Schedule

Ongoing cycles that continue after initial implementation. Post in the medication area alongside the Appendix E emergency reference.

Frequency	Task	Owner	Manual Ref	Deadline / Trigger
Every dose	Seven Rights verification and immediate MAR documentation	YCW	§12, §6	At administration
Every shift change	Medication count; CDS two-person count; key transfer; MAR completeness check	Shift Leads	§11	Before outgoing shift departs
Daily	Medication fridge temperature log (when refrigerated meds in use)	YCW / PC	§9	Each day
Weekly	MAR review; PRN usage review (flag 3+ uses in 7 days)	PC	§3, §18	Set weekly day
Monthly	Medication inventory, expiry check, storage/temperature check, review checklist	PC	§32.1	Last business day
Monthly	MAR audit (prior month, every youth)	PC	§38.1	By the 10th
Monthly	Review PC MAR audit summary	PM	§38.1	By the 15th
Monthly	CDS register review; month-end CDS reconciliation (Appendix C)	PM / PC	§10; App C	Month-end
Monthly	EpiPen expiry/accessibility check; authorization and training compliance (100%)	PC	§23.2; §39	Checklist / by 5th
Quarterly	Medication incident trend report to ED; corrective action log review	PM	§31; §39	Quarter-end
Annual	Manual review and ED re-approval; staff re-authorization; MALM renewals; Appendix I re-signing	ED / PM / PC	§4.1	July 1, 2027
Per event	Medication error: respond (§29); report to PM in 2 hrs; PM co-sign in 24 hrs; RCA in 5 business days; MCFD same day if Category 1/2	All / PC / PM	§29–31	Timelines per §29
Per event	New medication order actioned within 24 hours; profile and MAR updated before first dose	PC	§3, §8	Within 24 hrs
Per event	Prescription renewals initiated at least 2 weeks before expiry	PC	§32.2	Renewal calendar

Implementation Completion Sign-Off

All PM checklist items complete — PM Signature / Date	
All PC checklist items complete — PC Signature / Date	
All staff readiness checklists complete — PM Signature / Date	
Implementation reviewed — Executive Director Signature / Date	